

An Autistic Student in Your Clinic

For family doctors, university health & mental-health services — caring for an autistic **university / college student** (roughly ages 18–25).

The one idea

Autism, through **monotropism**, means a student's attention locks into a few deep, intense "tunnels." University is a *massive* transition: new autonomy, less structure, open-plan everything, group work, social pressure and disrupted routines. Many autistic students thrived under the predictability of school and then **hit a wall** — burnout, anxiety or crisis in the first or second year. Their focused interests are a genuine strength; the system around them is usually the problem.

What you may see — and what's really going on

You may see...	What's often happening
Was "fine," now can't attend, bathe, eat, or function	Autistic burnout from the cumulative load of a high-channel, unpredictable environment
Anxiety, depression, sleep collapse, disordered eating	Common co-occurrences ; also signals of overwhelm, not just primary illness
Skips lectures, misses deadlines; seems "lazy"	Autistic inertia + executive load under chronic depletion
Diagnosed only now, in crisis	University is a common late-identification point ("lost generation")
Prefers email; avoids calls, eye contact, group settings	Single-channel, literal communication; written contact reduces the load

Try this

- **Offer online/phone or written-first appointments**; allow a supporter and a sensory kit.
- **Screen for autistic burnout** specifically — exhaustion, loss of previously-held skills, reduced tolerance to stimulus — and treat it as distinct from depression.
- **Reduce and structure**: advocate for accommodations via the university disability service (quiet exam space, schedule predictability, written instructions).
- **Ask about interests and recovery** — protect access; rest and "doing things in an autistic way" are part of recovery.
- **Plan continuity** through holidays and the transition to adult/graduate life.

Avoid this

- Defaulting to “typical student stress/anxiety” and missing burnout or a first autism identification.
- Recommending generic social-skills or “push-through” approaches that increase masking.
- Assuming engagement = eye contact and verbal fluency.

When to get more support

Screen for suicidality and severe burnout directly. Signpost the **university disability/discovery service**, mentoring, and autistic student communities. Consider a **temporary reduction in academic load** or interruption of studies as a legitimate, evidence-aligned recovery step — not failure. Address co-occurring ADHD, anxiety, sleep and GI conditions.

If the student is a woman

Women and marginalised-gender students are over-represented among late-identified students and often mask intensely. A “high-achieving, sociable-on-the-surface” presentation with private collapse is a key pattern — ask about the gap between public performance and private exhaustion.

About this guide

*AI-assisted, derived from this project's research drafts — the **Monotropism** brief and the **Lifespan Practitioner & Health-Care guide** (Parts 3.3–3.4 & 4). Uses identity-first language. **Informational — not medical advice or a diagnostic tool.** Fit it to the individual.*

Key sources. Murray, Lesser & Lawson (2005); Murray, F. (2023); Dwyer (2025); Raymaker et al. (2020); Pearson & Rose (2021); Knight (2021, *tunnels not tasks*); AASPIRE AHAT. Full citations are in the source drafts.